

D7 – Office Visit Note

P.A. Kim, MD | Family Medicine | 07 Nov 2009

Patient: Caduceus, Opus J. MRN: 004-7703 DOB: 14 Mar 1956 Allergies: NKDA

OFFICE VISIT NOTE

Patient: Opus J. Caduceus | MRN: 004-7703

Date: 11/07/2009 | Provider: P.A. Kim, MD

Visit Type: Established Patient / Follow-Up

Practice: Oak Harbor Family Medicine (TRICARE), Oak Harbor WA

CHIEF COMPLAINT: 6-week follow-up after Nissen fundoplication. Chronic condition management.

HISTORY OF PRESENT ILLNESS:

53-year-old active duty USN with longstanding GERD inadequately controlled on max-dose PPI. GI consultation completed with Dr. R. Abara, gastroenterology, Seattle. EGD demonstrated LA grade C esophagitis with hiatal hernia. Underwent open Nissen fundoplication on 22 Sep 2009 at Swedish Medical Center, Seattle. Now 6 weeks post-operative. Reports partial symptom improvement with persistent regurgitation. Tolerating diet. No dysphagia, no hematemesis, no melena.

Separately, patient also s/p outpatient functional endoscopic sinus surgery with nasal polypectomy earlier in 2009 (Dr. K. Halvorson, Puget Sound Otolaryngology Associates, Seattle) for chronic rhinosinusitis with nasal polyps. Recovered well from that procedure.

CURRENT MEDICATIONS:

- Omeprazole 40 mg PO daily
- Flunisolide nasal spray PRN
- Ibuprofen PRN (patient reports recent GI intolerance)

PHYSICAL EXAMINATION:

Vitals: HR 76 | BP 132/82 | Wt 87 kg

General: Post-operative recovery on track. NAD.

HEENT: Nasal mucosa improved post-polypectomy. No acute sinusitis.

CV: Regular rate and rhythm. No murmurs.

Lungs: Clear bilaterally.

Abdomen: Soft, non-tender. Laparoscopic port sites well-healed.

ASSESSMENT & PLAN:

1. GERD s/p Nissen fundoplication (22 Sep 2009, Dr. Abara, Swedish Medical Center) - 6 weeks post-op with partial symptom improvement. Some regurgitation persisting. Continue omeprazole 40 mg daily per GI recommendation. Follow up with Dr. Abara as scheduled.
2. Chronic rhinosinusitis s/p nasal polypectomy (2009, Dr. Halvorson, Puget Sound Otolaryngology) - recovered well. Continue flunisolide nasal spray PRN for maintenance.
3. Bilateral knee pain - unchanged R > L. Patient reports GI intolerance to ibuprofen. Discontinue ibuprofen. Start naproxen 500 mg PO BID (with food). Counseled re: GI risks. Reassess in 3 months.

Follow-up: 6 months for chronic condition management.

Electronically signed by: P.A. Kim, MD

Date: 11/07/2009 10:42

